

IIH: vision, pressure and follow-up

CLINICAL TYPE B | Focused decisions for M3-M5

After diagnosis, protect the visual pathway

Three aims with different treatment roles

CONFIRMED IIH: THREE PARALLEL AIMS

Modify the disease
Sustained weight management

Protect vision
Pressure treatment + fields / OCT

Reduce headache morbidity
Treat the headache phenotype

Original management schematic for confirmed IIH after exclusion of secondary causes.

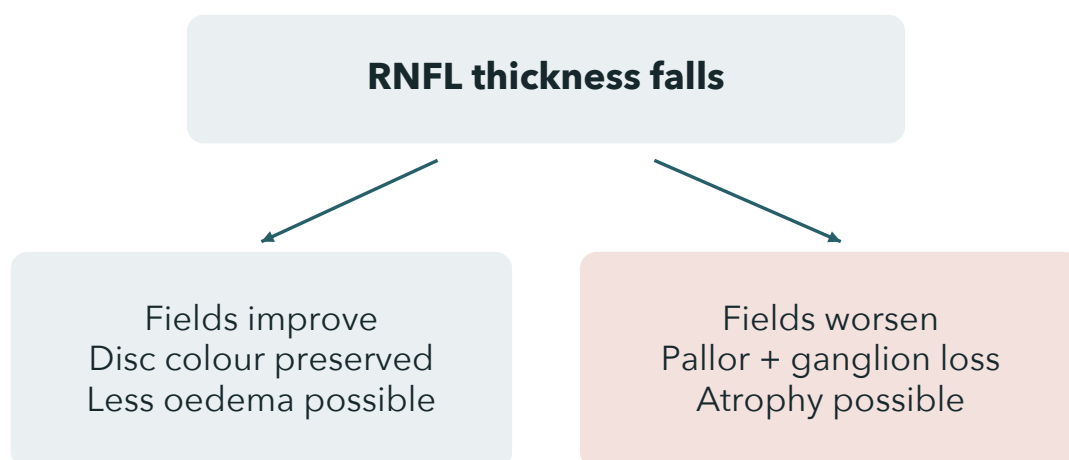
Do not substitute one aim for another

Finding	Clinical inference
Underlying disease	Sustained weight management is disease-modifying in typical obesity-associated IIH.
Raised pressure / papilloedema	Specialist pressure-lowering treatment and visual monitoring; declining function can require surgery.
Headache morbidity	Treat phenotype and medication overuse. Headache severity does not measure visual risk.

References Mollan et al.: IIH consensus guidance | NANOS: IIH and visual monitoring

A flatter disc is not automatically recovery

Interpret structure together with function



Interpret with visual function and the full trend

A flatter disc alone is not an outcome measure

Original schematic. Falling nerve-fibre thickness has two clinically different interpretations.

Review the trend

- 1** Compare reliable fields, acuity, disc colour and photographs/OCT, including ganglion-cell measures.
- 2** Less swelling + improving function may indicate recovery. Less swelling + pallor, ganglion-cell loss and worse fields suggests atrophy.
- 3** Expedite assessment for worsening visual function; urgent vision-preserving surgery may be needed. Serial LP is not routine definitive long-term care.